

COPAN INJECTION 1 ML AMP

DESCRIPTION:

Clear, colourless solution.

COMPOSITION:

Each ampoule contains Hyoscine Butylbromide 20 mg per ml.

PHARMACODYNAMICS:

It is an antispasmodic. It is a powerful smooth muscle relaxant, effective when given by mouth, by injection or by suppository. Hyoscine acts principally on the intramural parasympathetic ganglia in the smooth muscle of hollow abdominal organs.

In the recommended dosages, hyoscine butylbromide relieves smooth muscle spasm rapidly. Undesirable "atropine-like" side effects such as blurred vision, palpitation or dry mouth are rare.

PHARMACOKINETICS:

Upon oral administration, hyoscine butylbromide is quickly absorbed by the tissue and concentrates especially in the tissue of the gastrointestinal tract, liver and kidneys. The high affinity of this agent to the tissue is reflected by the very short half-life ($t_{1/2}$) of 3 min (distribution phase) of the blood levels, while the excretion rates are slow. Thus, in spite of the extremely low blood levels measurable over a short period of time, hyoscine butylbromide remains available at the site of action in the tissue in high concentrations.

INDICATION: Spasm of the gastrointestinal tract, biliary spasm, renal spasm, diagnostic aid in radiology.

RECOMMENDED DOSAGE:

1 or 2 ampoules (20 or 40 mg) THREE to FOUR times DAILY by intramuscular or slow intravenous injection. A maximum daily dose of 100mg should not exceed.

ROUTE OF ADMINISTRATION: For i.m. or slow i.v. injection.

CONTRAINDICATIONS:

Parenteral use is contra-indicated in glaucoma, prostatic hypertrophy with urinary retention, stenotic lesions of the gastrointestinal tract, tachycardia and megacolon. Copan Injection should not be administered to patients with tachycardia.

Risk-benefit should be considered when the following medical problem exist :-

- gastrointestinal/urinary obstruction
- myasthenia gravis
- ulcerative colitis
- paralytic ileus
- intestinal atony
- serious hepatic/renal disease

WARNINGS & PRECAUTIONS:

Copan Injection can cause tachycardia, hypotension and anaphylaxis, therefore use with caution in patients with cardiac conditions such as cardiac failure, coronary heart disease or cardiac arrhythmia and patients with cardiovascular disease (e.g. acute myocardial infarction, hypertension and conditions associated with tachycardia or hypertension, and in cardiac surgery). Monitoring of these patients is advised. Emergency equipment and personnel trained in its use must be readily available.

PRECAUTIONS TO CONSIDER:

Paediatrics: For all anticholinergics - Infants and young children are especially susceptible to the toxic effects of anticholinergics. Close supervision is recommended for infants and children with spastic paralysis or brain damage since an increased response to anticholinergics has been reported in these patients and dosage adjustments are often required.

When anticholinergics are given to children where the environmental temperature is high, there is risk of a rapid increase in body temperature because of these medications' suppression of sweat gland activity. A paradoxical reaction characterized by hyperexcitability may occur in children taking large doses of anticholinergics.

Geriatrics: Geriatric patients may respond to usual doses of anticholinergics with excitement, agitation, drowsiness or confusion.

Geriatric patients are especially susceptible to the anticholinergic side effects such as constipation, dryness of mouth, and urinary retention (especially in males). If these side effects occur and continue or are severe, medication should probably be discontinued.

Caution is also recommended when anticholinergics are given to geriatric patients, because of the danger of precipitating undiagnosed glaucoma.

Memory may become severely impaired in geriatric patients, especially those who already have memory problems, with the continued use of anticholinergics since these drugs block the actions of acetylcholine which is responsible for many functions of the brain, including memory functions.

Dental: Prolonged use of anticholinergics may decrease or inhibit salivary flow thus contributing to the development of caries, periodontal disease, oral candidiasis, and discomfort.

INTERACTION WITH OTHER MEDICAMENTS:

CNS depressants - increase sedation

Antacids/ antidiarrheals - decrease absorption

USE IN PREGNANCY & LACTATION:

Use in pregnancy: Safety during pregnancy has not been established, however, limited preclinical data have shown no hazard. The usual precautions regarding the use of medication at this time, especially during the first trimester, should be observed.

Use in lactation: Safety during lactation has not been established.

Pregnancy/Reproduction: For scopolamine: Scopolamine crosses the placenta. Studies with scopolamine have not been done in either animals or humans.

Labor- For scopolamine: Parenteral administration of scopolamine before the onset of labor may cause CNS depression in the neonate and may contribute to neonatal hemorrhage due to reduction in vitamin K-dependent clotting factors in the neonate.

Breast-Feeding: For all anticholinergics- Anticholinergics may inhibit lactation.

SIDE EFFECTS:

In the recommended dosage, side effects rarely occur. High dosage may produce side effects, such as dry mouth, blurred vision, transient impairment of accommodation and tachycardia. In some cases hypersensitivity reactions such as urticaria, rashes, facial and periorbital swelling may occur. Following parenteral administration (ampoules) very rare cases of shock have been observed.

Very rarely reports following parenteral administration of coma, hallucinations, dystonia, confusion, agitation and dizziness from which the patient recovered after drug withdrawal and appropriate treatment. In very rare cases, dyspnoea has been reported.

Immune system disorders

Not known: anaphylactic shock including cases with fatal outcome, anaphylactic reactions.

Cardiac disorders

Common: tachycardia.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Symptoms: Signs of poisoning following acute overdosage have not been observed in man. Single oral doses of up to 590 mg and quantities of the active substances of up to 1090 mg within 5 hours merely produced dry mouth, tachycardia, slight drowsiness and transient visual disorders.

Treatment: In the case of oral poisoning, gastric lavage with animal charcoal followed by magnesium sulfate (15%). Symptoms of overdosage respond to parasympathomimetics. For patients with glaucoma, pilocarpine is given locally. Boost circulation with sympathomimetics. For coma, central analeptics. Neuroleptics for mental excitation. In the case of respiratory paralysis, intubation, artificial respiration. In the case of orthostatic fall in blood pressure, it is sufficient for the patient to lie flat. Catheterisation in the case of urine retention.

INCOMPATIBILITIES: None known

STORAGE CONDITIONS:

Store below 30°C. Protect from light.

KEEP OUT OF REACH OF CHILDREN. *JAUHKAN DARIPADA KANAK-KANAK.*

SHELF LIFE:

Please refer to outer package.

PACK SIZE:

Packs of 10 ampoules x 1 ml only.

PRODUCT REGISTRATION HOLDER & MANUFACTURER:

DUOPHARMA (M) SDN BHD

Lot 2599, Jalan Seruling 59 Kawasan 3,

Taman Klang Jaya, 41200 Klang, Selangor, MALAYSIA.